

Disclosure of Unanticipated Outcomes

Policy Number	POL-PC-067	Owner / Department	Risk Management / Medical Staff
Effective Date	2023-08-15	Version	2.1
Last Revised	2026-03-14	Next Review	2028-03-14
Approved By	Patient Safety & Quality Committee	Applies To	All licensed providers and staff involved in a patient's care; coordinated by Risk Management

1. Purpose

To ensure that patients and families are told, honestly and compassionately, about unanticipated outcomes of care, consistent with The Joint Commission standard requiring that patients be informed of outcomes of care, including unanticipated outcomes.

2. Scope

Applies to any outcome that differs significantly from the anticipated result and that affects the patient, whether or not an error occurred. Disclosure is a clinical communication with the patient/family; it is distinct from the confidential event report filed under POL-RM-013.

3. Definitions

- **Unanticipated outcome** — a result of care that differs significantly and unexpectedly from the anticipated outcome.
- **Disclosure** — the communication to the patient/family of the facts of what happened, with empathy and ongoing care planning.
- **Event report** — the confidential quality-improvement record filed per POL-RM-013; it is not part of the medical record and is not the disclosure.

4. Who Discloses and When

IMPORTANT — Disclosure is made promptly — generally within 24 hours of recognizing the outcome — once the patient is stable and the immediate facts are known. The attending or responsible provider leads the conversation; Risk Management is notified first and helps coordinate, but disclosure is not delayed pending a complete investigation.

- The attending/responsible provider leads; a second support person (e.g., nurse manager or Risk Management) may attend.
- Notify Risk Management before or as soon as the disclosure occurs so it can coordinate and support.
- Hold the conversation in a private setting, in language the family understands, using a qualified interpreter where needed.

5. What Is Said

- State the known facts of what happened and what it means for the patient's care going forward.
- Express genuine empathy and, where appropriate, an apology for the harm or the experience.
- Do not speculate about cause, assign or admit blame, or point to other individuals before the facts are known.
- Describe the next steps, who to contact, and that the facility will follow up as more is learned.

6. Documentation and Distinction from the Event Report

Document the disclosure conversation in the medical record: who was present, the date and time, the facts communicated, the patient/family response and questions, and the plan for follow-up. Record only facts — not speculation, opinion, or the contents of the confidential event report. The event report itself is filed separately under POL-RM-013 and is neither placed in nor referenced from the medical record.

7. Severity & Reporting

The underlying event is classified and reported under POL-RM-013 according to its harm. A SEV-1 (sentinel-level) unanticipated outcome requires notification of the attending provider, Nursing Supervisor, Risk Management, and the Administrator-on-Call immediately and within 1 hour, and is managed per POL-RM-004; SEV-2 within 4 hours; SEV-3 or near miss within 24 hours. The disclosure obligation applies to any significant unanticipated outcome regardless of severity, and the disclosure is logged separately from the confidential event report.

8. References

- The Joint Commission, standard on informing patients of unanticipated outcomes of care (Rights and Responsibilities / Patient Rights).
- CMS Conditions of Participation, 42 CFR 482.13 — Patients' Rights.
- Related: POL-RM-004 (sentinel events), POL-PC-063 (code status), POL-PC-066 (informed consent), POL-RM-013 (severity & reporting).